

إسم المريض رباعياً : زهره سيد احمد طلبه
المحافظة : القاهرة
الحى / المدينة : المطرية
رقم البطاقة : 27004261600461 رقم التليفون : 01149018574
العمر(تاريخ الميلاد) : 26-04-1970 المهنة : بدون عمل
العنوان : 3ح بيوت العرب
تاريخ تسجيل الإستمارة : 20-09-2026

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التعليق على التشخيص : -----

GR I-E

رستم خیل

لدى حسين رمضان

صحة الايام الازرقعة

للجنة و مدير

كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

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ملاحظات :

رئيس
المجلس الطبي المتخصص

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Name: زهره سيد احمد طلبة National ID: 27004261600461	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





MDT form

1- Patient ID
1- NUMBER

2- Patient Name

Sex

3- Medical number
Female

4- Medical number
0113639

5- Date of birth

6- Date
215-2024

7- Date of birth

8- Patient age: 54 Y



Family history

9- Family history
+VE

Phone

10- Phone

01145016574

11- BMI

12- BMI

13- Reason of MDT

14- Reason of MDT
Not known

15- Medical status

16- Medical status
Not known

17- NRS Score

18- NRS Score

19- Group

20- Group

21- Date of MDT

22- Date of MDT

10/03/2024

23- Comorbidity

24- Comorbidity
diabetes
cardiac

25- Microscopic status

26- Microscopic status
Not known

27- Initial stage

28- Initial stage

Clinical data

29- Breast Imaging:

0

30- Metastasis:

31- Metastasis:
No cap without dyo.free
PAUS:free

32- DX
IDC

33- ER
A/B

34- PR
7/8

35- Her 2 Neu
Not known

36- Stom
Not known

37- R162
25

38- T
3

39- N
1

40- M
0

41- Sonomammography

42- Breast multicentric lesion with pathological axillary LNs(more than 3)

43- Patient history

44- received NACT AC-taxotere with good clinical response

45- RT MRM: free margin

46- LN 2/14, RCB II

MDT OUTCOME & PLAN

47- PORT, AT for 5-7 years, zometa every 6 months

SIGNATURE

48- MEDICAL ONCOLOGY

49- Signature

50- RADIOLOGY

51- Signature

52- SURGERY

53- Signature

54- PATHOLOGY

55- Signature

56- Psychiatric

57- Signature

58- Nutrition

59- Signature

Patient Name : الأستاذة / زهره سيد احمد طلبه العاصي
Patient No. : 27004261600461
Path No. : 103/24
Sex : Female
Age : 53 Y
Referring Doc :
Request No. : 13007542
Request Date : 08/01/2024
Verification Date : 22/01/2024
Financial Category : مبادره - اورام دار السلام
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 8/1/2024, 11:45 am.

Gross Examination:

- Multiple grayish white tissue cores up to 2 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: No.
- * Perineural invasion: No.
- * Angioinvasion: No.
- * IPT (Positive control showed appropriate reactivity):
 - ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8).
 - PR: Strong nuclear reaction in 60% of tumor cells (Allred score 7/8).
 - Her2: Faint incomplete membrane reaction in >10% of the tumor cells (score 1+).
 - Ki67: 25% nuclear reaction of tumor cells.

Diagnosis:

Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 7/8).

Verified by :

DR.Menna Nabil

Printing Date : 22/01/2024 12:14:37

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Printing By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadima- Cairo, Egypt



Patient Name : الأستاذة / زهره سيد احمد طلبه العاصي

Request No. : 13007542

Patient No. : 27004261600461

Request Date : 08/01/2024

Path No. : 103/24

Verification Date : 22/01/2024

Sex : Female

Financial Category : اورام نادر السلام - مبادره

Age : 53 Y

Department : Out Patient

Referring Doc :

Registered by : .marwa mohamed youss

Pathology Report

- Her2: Negative (score 1+).
- Ki67: Positive in 25% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr. Rana Fathy.

Dr. Toka Medhat.

Verified by :

DR.Menna Nabil

Printing Date : 22/01/2024 12:14:37

Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadima - Cairo, Egypt



Patient Name: Zahra Sayed Ahmed Tolba Al Assy	Date : 22/01/2024
Exam : ultrasound ABDOMEN& PELVIS	Age : 53 Year(s)
Medical ID: 215/2024	Pat. ID: 45245

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *enlarged in size (18cm at Rt MCL) bright parenchymal echopattern with smooth surface.*
- *No hepatic focal lesions or intra hepatic billiary radicle dilatation.*
- *Portal vein is patent and not dilated.*

Gall Bladder: -

- *Average distention and capacity with no stones or mud*
- *The wall is not thickened.*
- *CBD is not dilated.*

Spleen: -

- *Mildly enlarged in size 14cm , homogenous.*

Kidneys: -

- *Both are average in size, shape and echogenicity*
- *Suspected two pelvicalyceal system of the right kidney*
- *No stones or dilated pelvicalyceal system*
- *No gross lesion*

Para-aortic and pancreatic area: *show no gross abnormalities.*

No ascites

Urinary Bladder: -

- *Average capacity with no stones or diverticular out pouching*

Uterus and adenexal ; *no abnormalities could be detected*

Opinion:

Bright hepatomegaly likely fatty

Mild splenomegaly

Colonic gaseous distention

Signature Dr . Nermien Ramadan

22/01/2024 3:30 PM



Patient Name: Zahra Sayed Ahmed Tolba Al Assy	Date : 20/12/2023
Exam : MAMMOGRAPHY BOTH	Age : 53 Year(s)
Medical ID: 27004261600461	Pat. ID: 45245

Combined bilateral breast mammography and complementary ultrasound

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO-DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- Scattered dense glandular density (ACR B)
- Right edema pattern (increased skin thickening and coarse trabeculation)
- Right breast retro areolar and LOQ deeply seated spiculated mass lesions.
- No left suspicious microcalcifications,

Complementary ultrasound :

- Right breast 5 o'clock para areolar large heterogenous hypoechoic highly vascular mass lesions seen, surrounding by desmoplastic reaction which infiltrating nipple measuring about 34x30mm.
- Another Right 7 o'clock two ill-defined heterogenous hypoechoic lesion measuring about 13x12mm and 19x20mm.
- No left suspicious mass lesion.
- Normal skin and nipple areolar complex of left breast.
- Left nonspecific axillary lymph nodes.
- right multiple pathological axillary lymph nodes (more than 3 nodes) seen at level I, , the largest one measuring about 18x16mm with irregular in shape and absent hilum.
- Free supra and infra clavicular lymph nodes.

Opinion:

- right breast multicentric malignant looking lesions with ipsilateral pathological axillary lymph nodes at level I (more than 3 nodes), for histopathological correlation. (BIRADS 5).
- Left breast benign finding BIRADS 2.

Signature Dr. Nehal Hamdy

20/12/2023 3:03 PM



Total DLP (mGy.cm):

Patient Name: Zahra Sayed Ahmed Tolba Al Assy	Date : 22/01/2024
Exam : C.T CHEST	Age : 53 Year(s)
Medical ID: 215/2024 DLP:933	Pat. ID: 45245

- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- No suspicious osseous lesions.

Opinion:

- ***Right breast malignant looking lesions as described with ipsilateral pathological axillary lymph nodes.***
- ***No evidence of pulmonary, hepatic or osseous metastatic lesions on noncontrast basis.***

Dr. Ali Saber, MSc.

Signature Prof. Menna Tallah Hanafy

23/01/2024 02:43 م